The SozoRock Foundation

Economic research

Health access, workforce capacity, and AI & society

Economic conditions outlook, October 2026

Higher health costs and changing digital jobs complicate a growing economy. AI use is expanding faster than employees report clear plans for its use.

By Dr Oluwabiyi Adeyemo

October 2026

The US economy continued to expand in the first half of 2026. Institutions nevertheless face difficult decisions about health benefits, digital work, and artificial intelligence. Spending and adoption can advance while people still face barriers to using the services and opportunities they create.

Health coverage is one example. The latest Federal Reserve household survey found that 26 percent of adults skipped medical treatment because of cost in 2025. KFF's newly released employer survey shows workers paying more toward family coverage in 2026, with a substantially larger contribution at smaller firms. The surveys describe different populations and periods; together, they distinguish the cost of maintaining coverage from the cost of obtaining care.

Digital work presents a different challenge. The Bureau of Labor Statistics projects growth in security and network architecture roles alongside a decline in systems administration. Training priorities therefore depend on the occupation and the work involved. Meanwhile, Gallup's August 2026 data show 53 percent of employees using AI at least occasionally, but only 26 percent reporting a clearly communicated organizational plan.

Over the next six to twelve months, three decisions will shape the outlook: how employers distribute higher benefit costs, how training connects people to changing jobs, and how organizations make AI-supported services accountable to their users. This national analysis examines those choices across The SozoRock Foundation's work in health access, workforce capacity, and AI and society.

Workers at smaller firms contribute about 42 percent more toward family coverage.

KFF, 2026 Employer Health Benefits Survey; SozoRock calculation. See Exhibit 3.

This outlook combines published national statistics and surveys. Each exhibit identifies its population, period, and source. The closing assessment sets out conditions to monitor through October 2027.

Domestic demand strengthened as headline growth eased

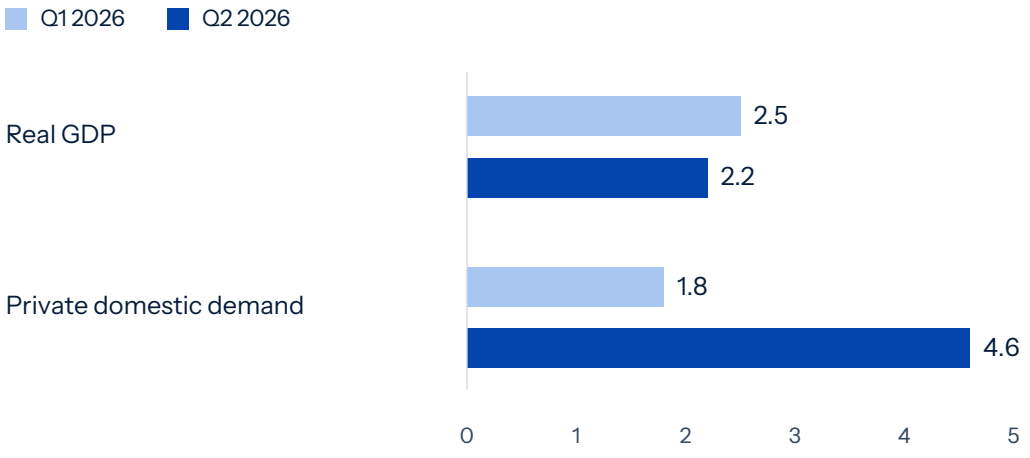
Real gross domestic product grew at a 2.2 percent annualized rate in the second quarter of 2026, following a revised 2.5 percent in the first. Consumer spending and private fixed investment, measured together, accelerated from 1.8 to 4.6 percent. That contrast matters: the headline slowdown did not reflect a broad retreat in private domestic demand.

GDP includes trade, government spending, and inventory changes; final sales to private domestic purchasers isolates consumption and fixed investment. Its stronger reading supports a firmer assessment of domestic demand than the GDP comparison alone. Neither measure describes how gains are distributed among households.

Exhibit 1

Private domestic demand accelerated despite slower GDP growth.

Real growth from the preceding quarter, annualized and seasonally adjusted, %



Private domestic demand means real final sales to private domestic purchasers: consumer spending plus gross private fixed investment. Series overlap and must not be added. Q1 values are revised; all figures are from the third-estimate release.

Source: Bureau of Economic Analysis, GDP third estimate, September 30, 2026.

For the year ahead, the question is whether demand can remain strong without further pressure on household budgets. The personal consumption expenditures price index rose at a 5.0 percent annualized rate in the second quarter, up from 4.2 percent in the first. Those are quarterly annualized rates, not twelve-month inflation readings.

The improvement to watch is sustained real spending growth alongside slower price increases. If spending weakens while essential costs keep rising, households have less room to absorb higher health contributions.

Insurance leaves a substantial affordability gap

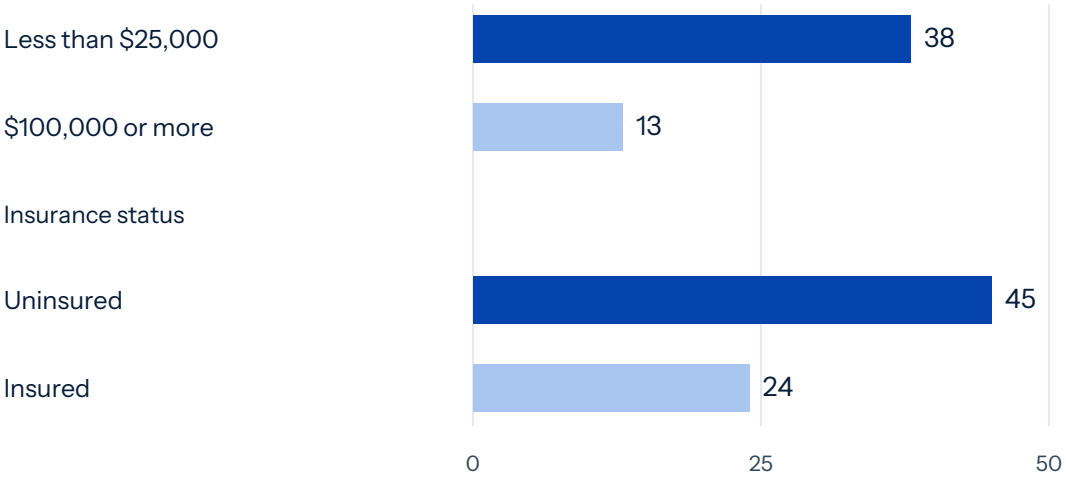
The share of adults reporting cost-related missed care fell from 28 percent in 2024 to 26 percent in 2025. The larger divide was between income groups: 38 percent of adults with family income below $25,000 skipped care, compared with 13 percent at $100,000 or more. That is a 25-percentage-point gap.

Coverage offers another view. Forty-five percent of uninsured adults reported missed care because of cost, as did 24 percent of insured adults. An enrollment measure alone would therefore overlook an important part of the access problem. These are overlapping population groups, not estimates of what insurance or an income increase would cause.

Exhibit 2

Cost-related missed care differs sharply by income and insurance.

Skipped medical treatment because of cost in the prior 12 months, 2025, % of US adults within each group



Weighted survey estimates. Income brackets shown are the two endpoints, not the full distribution. Income and insurance groups overlap. Survey fielded October 17–28, 2025; final analytical sample: 12,934 US adults.

Source: Federal Reserve Board, Economic Well-Being of US Households in 2025, May 2026.

Federal Reserve Board, survey methods.

The practical distinction is between paying for coverage and paying when treatment is needed. A benefit can be available on paper while the premium, deductible, or a particular service remains unaffordable. The survey identifies cost-related missed care; it does not identify which financial barrier caused each missed visit.

For access programs, the next step is to connect assistance to a defined service and follow whether care is completed. Eligibility, appointment attendance, and the patient's remaining payment belong in the same assessment. A reduction in reported cost barriers among participants would be more informative than enrollment alone.

Workers at smaller firms pay more for family coverage

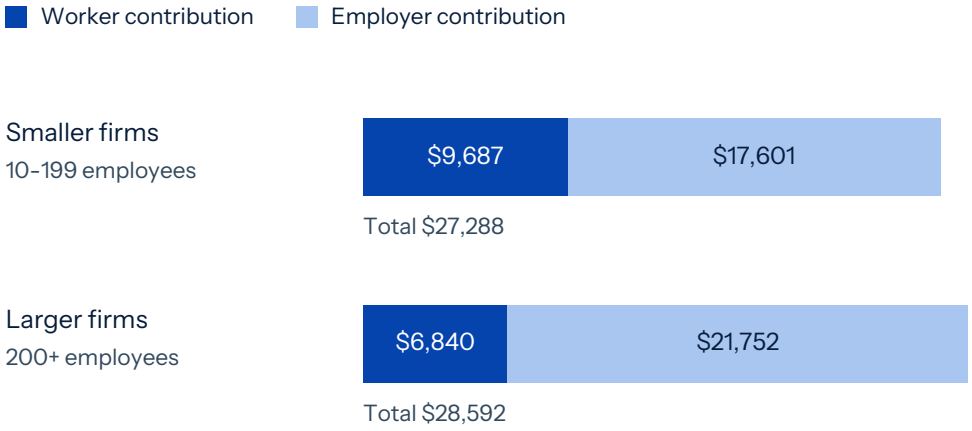
Workers with employer-sponsored family coverage contributed an average of $7,471 toward annual premiums in 2026, up $621 from 2025, or about 9 percent. The total premium, including the employer contribution, averaged $28,303. These premiums are paid to maintain coverage; deductibles and other charges for care are additional.

The worker contribution was $9,687 at firms with 10 to 199 employees, versus $6,840 at firms with 200 or more. The $2,847 difference is about 42 percent of the larger-firm amount. KFF reports lower total family premiums at smaller firms, so the larger employee payment reflects how the bill is divided, not simply a higher total premium.

Exhibit 3

Workers at smaller firms pay $2,847 more toward family premiums.

Average annual contribution for covered workers with family coverage, 2026, US dollars



Calculated worker-contribution difference: $2,847 (41.6%).

Employer contributions are calculated as total premium minus worker contribution. Estimates describe covered workers, not all employees. Survey: 1,937 nonfederal public and private employers with 10+ workers, fielded January-July 2026; overall response rate 11%.

Source: KFF, 2026 Employer Health Benefits Survey, Figures C and 6.3, October 22, 2026.

The 2027 renewal decision is consequently about both price and distribution. An employer can absorb an increase, pass more to workers, change benefits, or renegotiate purchasing arrangements. Each option moves costs or changes access; a lower employer bill is not automatically a lower household burden.

The affordability test compares the employee's annual premium and expected cost sharing with pay, alongside continued access to essential providers. For purchasers, this means evaluating each renewal option against both the household bill and service availability. A cheaper plan that removes a needed provider creates a different access problem.

Digital jobs have different growth prospects

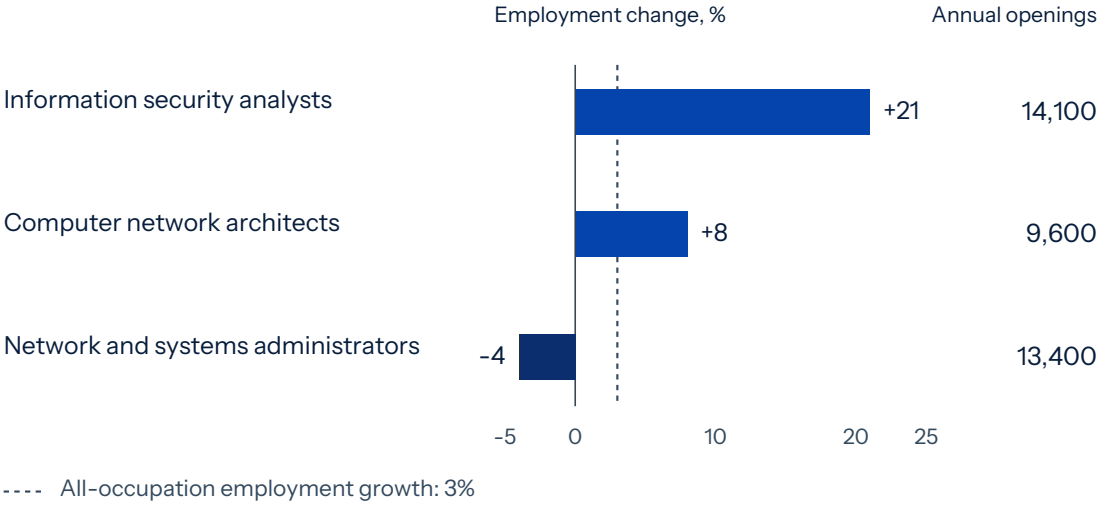
The Bureau of Labor Statistics projects employment growth of 21 percent for information security analysts and 8 percent for computer network architects from 2025 to 2035. Network and computer systems administrators face a projected 4 percent decline. The economy-wide benchmark is 3 percent growth.

Systems administration still has 13,400 projected annual openings from replacement needs, close to the 14,100 openings in information security. Employment growth and hiring needs are different measures. Retirement and occupational transfers create openings even in a shrinking occupation.

Exhibit 4

Security has stronger projected growth; other roles still have openings.

US employment projections, 2025-35: total percentage change and average annual openings



BLS ten-year projections, not current vacancies or a forecast for the next twelve months. Growth percentages are rounded. Annual openings include occupational transfers and labor-force exits. All occupations: 3% projected employment growth.

Source: US Bureau of Labor Statistics, Occupational Outlook Handbook, 2025-35 projections: information security analysts; computer network architects; network and computer systems administrators.

For cloud, security, and AI training, the distinction argues for occupation-specific preparation. Network architecture involves design and deployment; security roles include investigation and protection of systems. BLS identifies automation and changes in infrastructure delivery as pressures on administration work.

Over the next year, employers and training providers can test this direction through actual vacancies, paid project placements, and retention after entry. Supervised work in incident investigation, access control, and network design offers a concrete way to assess capability. Success means sustained employment and useful work, not simply more course completions.

AI use is advancing faster than organizational direction

Gallup's August 2026 data show that 53 percent of US employees used AI in their role at least a few times a year, up from 40 percent in May 2025. Over the same period, the share saying their organization had communicated a clear AI plan increased from 22 to 26 percent. Use rose by 13 percentage points; reported communication rose by four.

This is a gap between two employee-reported measures. It does not identify how many users lack a plan, because the published figures do not cross-tabulate the answers. The finding nevertheless raises an operational question: are institutions setting expectations for tools that employees are already using?

Exhibit 5

Employee AI use has grown faster than reports of a clear AI plan.

US employees reporting each measure, %

Use means at least a few times a year. Plan means employees report their organization communicated a clear AI integration plan. Separate questions, not mutually exclusive groups. Weighted Gallup Panel surveys of US employees aged 18+; month labels follow the source's tables.

Source: Gallup, Global Indicator: Artificial Intelligence, data tables through August 2026.

The next stage is deciding which tasks AI can support, who reviews the output, and which information can enter the tool. A clear plan makes those responsibilities explicit. Training then has a specific purpose: employees learn to complete a defined task and recognize when the result requires correction or escalation.

For the next twelve months, stronger implementation would show up in fewer corrected errors, shorter completion times, and consistent service quality within approved workflows. Reported adoption remains useful, but it is an input measure. A growing user count cannot by itself establish that an organization has improved its performance.

Public concern makes accountability part of AI deployment

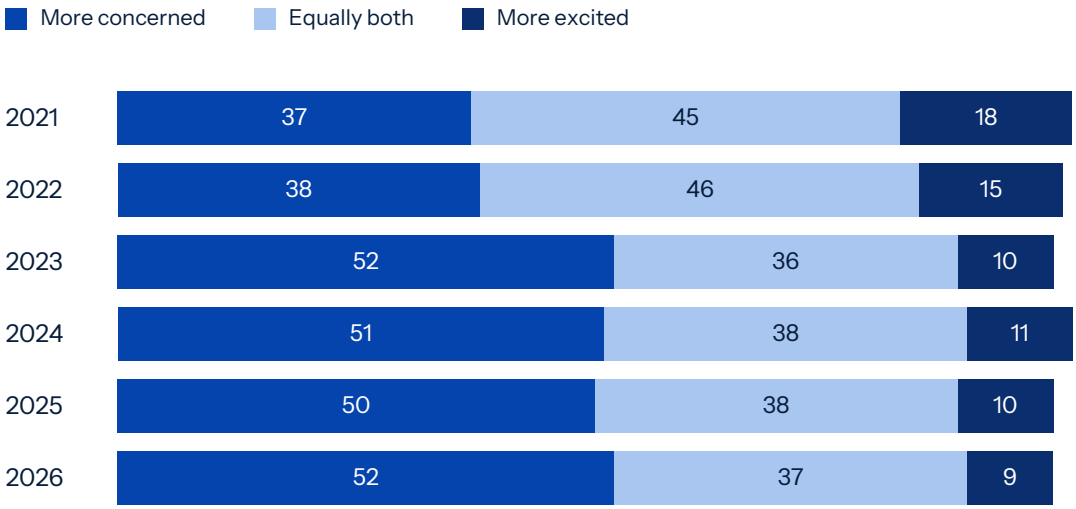
In Pew Research Center's June 2026 survey, 52 percent of US adults were more concerned than excited about AI's growing role in daily life. The share was 37 percent in 2021, but most of its rise occurred by 2023. Concern has since remained near half of adults.

The employment question adds context. Seventy-one percent expected AI to lead to fewer US jobs over the next twenty years. That measures a public expectation, not a labor-market forecast. Organizations introducing AI into work, education, or care therefore face questions about whose interests the system serves and how affected people can challenge a result.

Exhibit 6

Concern about AI has stayed near half of adults since 2023.

US adults' feelings about increased AI use in daily life, %



Categories: more concerned than excited; equally concerned and excited; more excited than concerned. Nonresponses are not shown; totals may be below 100%. Latest wave: June 22-28, 2026, n = 3,488 US adults; weighted American Trends Panel.

Source: Pew Research Center, Young adults in the US are increasingly wary of AI, August 18, 2026.

Pew Research Center, historical data table.

Residents and workers can identify consequences a procurement checklist misses, such as an inaccessible appeal route or uncertainty about who owns a decision. Institutions can then publish the permitted use, the responsible person, and the process for correcting an error.

The test over the coming year is whether those arrangements work. Resolution time, repeat complaints, corrected decisions, and participation across affected groups offer more useful feedback than attendance at a forum alone. Consultation creates an opportunity to improve a service; trust must still be earned through its operation.

The next year turns on three institutional choices

The SozoRock Foundation's assessment for the next six to twelve months is that benefit affordability, occupational transitions, and accountable AI use will remain central tests of progress. The following conditions distinguish improvement from deterioration; they are scenarios for monitoring, not probability-weighted forecasts.

Health access

Employers and health purchasers: protect access when renewing benefits.

Improvement: Worker payments stabilize relative to pay, essential providers remain available, and cost-related missed care falls. Pressure: Higher contributions or narrower coverage shift costs to households and interrupt treatment.

Watch: Worker premium and cost sharing; completed care; reported cost barriers.

Workforce capacity

Employers and training providers: connect learning to a specific job and paid work.

Improvement: Supervised projects lead to placement, retention, and demonstrated capability in security and infrastructure work. Pressure: Training expands while placements stall or the curriculum misses changing tasks.

Watch: Vacancies by occupation; paid placements; six-month retention.

AI & society

Service leaders and public bodies: name responsibility before expanding AI-supported decisions.

Improvement: Clear rules and workable appeals accompany better task performance and fewer repeated errors. Pressure: Use expands while errors, unresolved complaints, or inaccessible services persist.

Watch: Corrected errors; completion time; appeal resolution; user experience.

The common test is the result people experience: completed care, sustained employment, and decisions they can understand and challenge. Those outcomes belong alongside spending and adoption in the next assessment.

About this analysis. Original SozoRock analysis of published US statistics and surveys; no original survey was conducted. Data checked October 27, 2026. Sources cover different periods and populations; estimates are not pooled. Ten-year occupational projections are distinguished from the near-term scenarios above. Survey comparisons are descriptive; causal effects are not estimated.

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